MOOD DISORDERS AND SUICIDE:

Understanding Depression, Mania, Bipolar Disorders, and Suicide Prevention

Prepared by:

JOEY C. LANDICHO, MSP, RPsy

Faculty

Learning Outcomes:

By the end of the lesson, students should be able to:

1. Differentiate depressive disorders from bipolar and related disorders.
2. Identify the defining characteristics and diagnostic distinctions of major depressive disorder, persistent depressive disorder, bipolar I disorder, bipolar II disorder, and cyclothymic disorder.
3. Explain biological, psychological, social, and cultural contributions to mood disorders.
4. Compare major biological and psychological treatment approaches.
5. Analyze suicide risk factors, warning signs, and protective factors.
6. Apply appropriate, evidence-informed principles of suicide prevention and referral.



Topic Outline:

PART A — UNDERSTANDING MOOD DISORDERS

- Depression and mania
- Structure of mood disorders
- Depressive disorders
- Bipolar and related disorders

PART B — PREVALENCE AND DEVELOPMENT

- Prevalence across the life span
- Cultural influences
- Mood disorders and creativity

PART C — CAUSES AND TREATMENT

- Biological, psychological, and social factors
- An integrative explanation
- Medications and brain stimulation
- Psychological and combined treatments
- Relapse prevention

PART D — SUICIDE

- Statistics and causes
- Risk and protective factors
- Suicide contagion



Icebreaker:
Is It a Mood or a Mood Disorder?

Consider these experiences:

- Feeling sad after receiving disappointing news.
- Feeling unusually energetic after a personal achievement.
- Experiencing low mood and loss of interest for weeks.
- Sleeping very little while feeling unusually energized and engaging in risky decisions.



Discussion question:

What distinguishes an ordinary emotional response from a possible mood disorder?

Depression and Mania: An Overview

Depression

- **Persistent low or depressed mood**, and/or diminished interest or pleasure.
- May involve changes in **sleep, appetite, energy, concentration, and self-evaluation**.
- Can **interfere significantly** with daily functioning.

Mania

- A distinct period of **abnormally elevated, expansive, or irritable mood**.
- **Increased energy** or activity.
- May involve **decreased need for sleep, rapid speech, racing thoughts, grandiosity, distractibility, or risky behavior**.
- Can cause **marked impairment**, require hospitalization, or involve psychotic features.



Key distinction: Mania is not simply happiness, enthusiasm, or productivity.



The Structure of Mood Disorders

Depressive disorders

- Major depressive disorder
- Persistent depressive disorder
- Other specified or unspecified depressive disorders

Bipolar and related disorders

- Bipolar I disorder
- Bipolar II disorder
- Cyclothymic disorder
- Other specified or unspecified bipolar and related disorders



Core distinction: A history of mania changes the diagnostic classification.

DEPRESSIVE DISORDERS

Major Depressive Disorder (MDD)



Clinical description

A major depressive episode involves at least five symptoms during the same two-week period, representing a change from previous functioning. At least one symptom must be:

- Depressed mood, or
- Markedly diminished interest or pleasure.

Other symptoms may include:

- Changes in sleep or appetite.
- Fatigue or loss of energy.
- Psychomotor agitation or retardation.
- Feelings of worthlessness or excessive guilt.
- Difficulty concentrating or making decisions.
- Recurrent thoughts of death or suicidal ideation.

Symptoms must **cause clinically significant distress or impairment** and be assessed in context.

Major Depressive Disorder (MDD)

Major depressive episode

Five (or more) of the following symptoms have been present during the same **2-week period** and represent a change from previous functioning; **at least one** of the symptoms must be either (1) depressed mood or (2) markedly diminished interest or pleasure.

Possible symptoms

- Depressed mood most of the day, nearly every day.
- Markedly diminished interest or pleasure in most activities.
- Significant weight loss or weight gain, or decreased or increased appetite.
- Insomnia or hypersomnia, nearly every day.
- Psychomotor agitation or retardation (observable by others).
- Fatigue or loss of energy, nearly every day.
- Feelings of worthlessness or excessive or inappropriate guilt.
- Diminished ability to think or concentrate, or indecisiveness.
- Recurrent thoughts of death, suicidal ideation without a specific plan, or a suicide attempt or specific plan.

Clinical requirements

1. Symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.
2. The symptoms are not attributable to the physiological effects of a substance or another medical condition.
3. The episode is not better explained by a schizoaffective disorder or other psychotic disorder.
4. There has never been a manic episode or a hypomanic episode (for MDD classification).

Note: Diagnosis depends on the full symptom pattern, duration, context, and clinical judgment.

MDD: Coding and Recording Procedures

The diagnostic code for major depressive disorder is based on whether this is a **single or recurrent episode**, **current severity**, presence of **psychotic features**, and **remission status**.

Specifier	Single episode	Recurrent episode
Mild	F32.0	F33.0
Moderate	F32.1	F33.1
Severe	F32.2	F33.2
With psychotic features	F32.3	F33.3
In partial remission	F32.4	F33.41
In full remission	F32.5	F33.42
Unspecified	F32.9	F33.9

Additional specifiers

The following specifiers may be used (as applicable to the current or most recent episode):

- With anxious distress
- With mixed features
- With melancholic features
- With atypical features
- With catatonia
- With peripartum onset
- With seasonal pattern (applies to recurrent episodes)

Note: Recurrent episodes require at least 2 consecutive months between episodes without full criteria for a major depressive episod
If psychotic features are present, use the psychotic-features specifier regardless of severity.



Persistent Depressive Disorder and Other Depressive Conditions

Persistent Depressive Disorder (PDD)

- Depressed mood for most of the day, more days than not.
- Persists for at least two years in adults and at least one year in children and adolescents.
- May involve low energy, poor concentration, low self-esteem, hopelessness, or sleep and appetite changes.

Other depressive conditions

- Premenstrual dysphoric disorder.
- Disruptive mood dysregulation disorder.
- Substance/medication-induced depressive disorder.
- Depressive disorder due to another medical condition.



Diagnosis depends on the specific symptom pattern, duration, and exclusions.

Persistent Depressive Disorder (PDD)

F34.1

This disorder represents a **consolidation of DSM-IV-defined chronic major depressive disorder and dysthymic disorder**.

Diagnostic criteria

A Depressed mood for most of the day, for more days than not, as indicated by either subjective account or observation by others, for at least 2 years.

Note: In children and adolescents, mood can be irritable and duration must be at least 1 year.

B Presence, while depressed, of two (or more) of the following:

1. Poor appetite or overeating.
2. Insomnia or hypersomnia.
3. Low energy or fatigue.
4. Low self-esteem.
5. Poor concentration or difficulty making decisions.
6. Feelings of hopelessness.

C During the 2-year period (1 year for children or adolescents) of the disturbance, the individual has never been without the symptoms in Criteria A and B for more than 2 months at a time.

D Criteria for a major depressive disorder may be continuously present for 2 years.

E There has never been a manic episode or a hypomanic episode.

F The disturbance is not better explained by a persistent schizoaffective disorder, schizophrenia, delusional disorder, or other specified or unspecified schizophrenia spectrum and other psychotic disorders.

G The symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hypothyroidism).

H The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of fun[illegible]ning.

Persistent Depressive Disorder: Specifiers

Major depressive episode during the 2-year period

- If criteria are sufficient for a diagnosis of a major depressive episode at any time during the 2-year period of depressed mood, then a separate diagnosis of major depressive disorder should be made in addition to the diagnosis of persistent depressive disorder, along with the relevant specifier (e.g., with intermittent major depressive episodes, with current episode).

Other specifiers

Specify if:

- With anxious distress
- With atypical features
- In partial remission
- In full remission
- Early onset (if onset is before age 21 years)
- Late onset (if onset is at age 21 years or older)

Specifiers for the most recent 2 years

Specify if (for most recent 2 years of persistent depressive disorder):

- **With pure dysthymic syndrome**: Full criteria for a major depressive episode have not been met in at least the preceding 2 years.
- **With persistent major depressive episode**: Full criteria for a major depressive episode have been met throughout the preceding 2-year period.
- **With intermittent major depressive episodes, with current episode**: Full criteria for a major depressive episode are currently met, but there have been periods of at least 8 weeks in at least the preceding 2 years with symptoms below the threshold for a full major depressive episode.
- **With intermittent major depressive episodes, without current episode**: Full criteria for a major depressive episode are not currently met, but there has been one or more major depressive episodes in at least the preceding 2 years.

Current severity specifiers

- Mild
- Moderate
- Severe



Other Depressive Disorders

- **Premenstrual Dysphoric Disorder (PMDD)** – Severe mood changes, irritability, anxiety, or depression occurring before menstruation and improving after it begins.
- **Disruptive Mood Dysregulation Disorder (DMDD)** – Persistent irritability and frequent, severe temper outbursts in children.
- **Substance/Medication-Induced Depressive Disorder** – Depressive symptoms caused by substance use, intoxication, withdrawal, or medication.
- **Depressive Disorder Due to Another Medical Condition** – Depression directly resulting from the physiological effects of an underlying medical condition.
- **Other Specified Depressive Disorder** – Depressive symptoms cause significant distress or impairment, but the person does not meet the full criteria for a specific depressive disorder, and the clinician explains why.
- **Unspecified Depressive Disorder** – Depressive symptoms cause significant distress or impairment, but there is insufficient information to specify the particular disorder or why the criteria are not met.
- **Specifiers for Depressive Disorders** – Describe specific features of depression, including anxiety, manic symptoms, severe loss of pleasure, mood reactivity, psychosis, catatonia, pregnancy-related onset, seasonal recurrence, and persistent symptoms.



Premenstrual Dysphoric Disorder (PMDD)

F32.81

A. Cyclical pattern and timing

- In the majority of menstrual cycles, at least **five symptoms** must be present in the **final week before the onset of menses**, start to *improve* within a few days after the onset of menses, and become *minimal* or *absent* in the week postmenses.

B. Core affective symptoms (at least one)

1. Marked affective lability (e.g., mood swings; feeling suddenly sad or tearful, or increased sensitivity to rejection).
2. Marked irritability or anger or increased interpersonal conflicts.
3. Marked depressed mood, feelings of hopelessness, or self-deprecating thoughts.
4. Marked anxiety, tension, and/or feelings of being keyed up or on edge.

C. Additional symptoms (one or more, to reach a total of five)

1. Decreased interest in usual activities (e.g., work, school, friends, hobbies).
2. Subjective difficulty in concentration.
3. Lethargy, easy fatigability, or marked lack of energy.
4. Marked change in appetite; overeating; or specific food cravings.
5. Hypersomnia or insomnia.
6. A sense of being overwhelmed or out of control.
7. Physical symptoms such as breast tenderness or swelling, joint or muscle pain, a sensation of “bloating,” or weight gain.

D. Functional impairment

- The symptoms cause clinically significant distress or interference with work, school, usual social activities, or relationships with others (e.g., avoidance of social activities; decreased productivity and efficiency at work, school, or home).

E. Differential diagnosis

- The disturbance is not merely an exacerbation of the symptoms of another disorder, such as major depressive disorder, panic disorder, persistent depressive disorder, or a personality disorder (although it may co-occur with any of these disorders).

F. Confirmation of symptom pattern

- Criterion A should be confirmed by prospective daily ratings during at least two symptomatic cycles. (**Note:** The diagnosis may be made provisionally prior to this confirmation.)

G. Rule out other causes

- The symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication, other treatment) or another medical condition (e.g., hyperthyroidism).

Disruptive Mood Dysregulation Disorder (DMDD)

F34.81

Core presentation

- Severe, recurrent temper outbursts manifested verbally or behaviorally (e.g., verbal rages, physical aggression) that are grossly out of proportion in intensity or duration to the situation or provocation.
- Outbursts are inconsistent with developmental level.
- Outbursts occur, on average, three or more times per week.
- Between outbursts, the mood is persistently irritable or angry most of the day, nearly every day, and observable by others (e.g., parents, teachers, peers).
- Symptoms have been present for 12 or more months, with no period of 3 or more consecutive months without all symptoms.

Onset and settings

- Age at onset is before 10 years.
- Diagnosis should not be made for the first time before age 6 years or after age 18 years.
- Criteria for temper outbursts and persistently irritable/angry mood are present in at least two of three settings (i.e., at home, at school, with peers) and are severe in at least one of these.

Diagnostic exclusions

- There has never been a distinct period lasting more than 1 day during which the full symptom criteria (except duration) for a manic or hypomanic episode have been met.
- The behaviors do not occur exclusively during a major depressive disorder and are not better explained by another mental disorder (e.g., autism spectrum disorder, PTSD, separation anxiety disorder, persistent depressive disorder).
- The symptoms are not attributable to the physiological effects of a substance or another medical or neurological condition.

Note: This diagnosis cannot coexist with oppositional defiant disorder, intermittent explosive disorder, or bipolar disorder, though it can coexist with other disorders. When criteria for both DMDD and oppositional defiant disorder are met, DMDD takes precedence.

Substance/Medication-Induced Depressive Disorder

Diagnostic Criteria

A **A prominent and persistent disturbance in mood that predominates in the clinical picture and is characterized by:**

- Depressed mood, or
- Markedly diminished interest or pleasure in all, or almost all, activities.

B **There is evidence from the history, physical examination, or laboratory findings that:**

1. The symptoms developed during or soon after substance intoxication or withdrawal or after exposure to or withdrawal from a medication.
2. The involved substance/medication is capable of producing the symptoms in Criterion A.

C **The disturbance is not better explained by a depressive disorder that is not substance/medication-induced. Evidence of an independent depressive disorder may include:**

- Symptoms preceded the onset of substance/medication use.
- Symptoms persist for a substantial period of time (e.g., about 1 month after the cessation of acute withdrawal or severe intoxication).
- A history of recurrent, non-substance/medication-related depressive episodes.

D The disturbance does not occur exclusively during the course of a delirium.

E The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.



Substance/Medication-Induced Depressive Disorder

Notes and coding considerations

- **Note:** This diagnosis should be made instead of a diagnosis of substance intoxication or substance withdrawal only when the symptoms in Criterion A predominate in the clinical picture and when they are sufficiently severe to warrant clinical attention.
- **Coding note:** The ICD-10-CM codes for the [specific substance/medication]-induced depressive disorders are indicated in the table. The ICD-10-CM code depends on whether or not there is a comorbid substance use disorder present for the same class of substance.
  - If a mild substance use disorder is comorbid, the 4th character is "1" (e.g., mild cocaine use disorder with cocaine-induced depressive disorder).
  - If a moderate or severe substance use disorder is comorbid, the 4th character is "2" (record "moderate" or "severe" [substance] use disorder, depending on severity).
  - If there is no comorbid substance use disorder (e.g., after a one-time heavy use), the 4th character is "9," and record only the substance-induced depressive disorder.

ICD-10-CM codes for substance/medication-induced depressive disorders

Substance/medication	With mild use disorder	With moderate or severe use disorder	Without use disorder
Alcohol	F10.14	F10.24	F10.94
Phencyclidine	F16.14	F16.24	F16.94
Other hallucinogen	F16.14	F16.24	F16.94
Inhalant	F18.14	F18.24	F18.94
Opioid	F11.14	F11.24	F11.94
Sedative, hypnotic, or anxiolytic	F13.14	F13.24	F13.94
Amphetamine-type substance (or other stimulant)	F15.14	F15.24	F15.94
Cocaine	F14.14	F14.24	F14.94
Other (or unknown) substance	F19.14	F19.24	F19.94

Onset specifiers

- **With onset during intoxication:** If criteria are met for intoxication with the substance and the symptoms develop during intoxication.
- **With onset during withdrawal:** If criteria are met for withdrawal from the substance and the symptoms develop during, or shortly after, withdrawal.
- **With onset after medication use:** If symptoms developed at initiation of medication, with a change in use of medication, or during withdrawal of medication.

Depressive Disorder Due to Another Medical Condition

Diagnostic Criteria

A A prominent and persistent disturbance in mood that predominates in the clinical picture and is characterized by depressed mood or markedly diminished interest or pleasure in all, or almost all, activities.

B There is evidence from the history, physical examination, or laboratory findings that the disturbance is the direct pathophysiological consequence of another medical condition.

C The disturbance is not better explained by another mental disorder (e.g., adjustment disorder, with depressed mood, in which the stressor is a serious medical condition).

D The disturbance does not occur exclusively during the course of a delirium.

E The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Coding and specifiers (ICD-10-CM)

Specify if:

- **F06.31 With depressive features:**
  Full criteria are not met for a major depressive episode.
- **F06.32 With major depressive–like episode:**
  Full criteria are met (except Criterion C) for a major depressive episode.
- **F06.34 With mixed features:**
  Symptoms of mania or hypomania are also present but do not predominate in the clinical picture.

Coding note:

Include the name of the other medical condition in the name of the mental disorder (e.g., F06.31 depressive disorder due to hypothyroidism, with depressive features). The other medical condition should also be coded and listed separately immediately before the depressive disorder due to the medical condition (e.g., E03.9 hypothyroidism; F06.31 depressive disorder due to hypothyroidism, with depressive features).

Other Specified Depressive Disorder (F32.89)

Overview

- This category applies to presentations in which symptoms characteristic of a depressive disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the depressive disorder diagnostic class and do not meet criteria for adjustment disorder with depressed mood or adjustment disorder with mixed anxiety and depressed mood.
- Used when the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific depressive disorder (record as "other specified depressive disorder" followed by the specific reason, e.g., "short-duration depressive episode").

Examples of presentations

1. **Recurrent brief depression**
   - Depressed mood and at least four other symptoms of depression for 2–13 days at least once per month (not associated with the menstrual cycle) for at least 12 consecutive months.
   - Presentation has never met criteria for any other depressive or bipolar disorder and does not currently meet active or residual criteria for any psychotic disorder.

Examples of presentations (continued)

2. **Short-duration depressive episode (4–13 days)**
   - Depressed affect and at least four of the other eight symptoms of a major depressive episode.
   - Associated with clinically significant distress or impairment that persists for more than 4 days, but less than 14 days.
   - Presentation has never met criteria for any other depressive or bipolar disorder, does not currently meet active or residual criteria for any psychotic disorder, and does not meet criteria for recurrent brief depression.
3. **Depressive episode with insufficient symptoms**
   - Depressed affect and at least one of the other eight symptoms of a major depressive episode.
   - Associated with clinically significant distress or impairment that persist for at least 2 weeks.
   - Presentation has never met criteria for any other depressive or bipolar disorder, does not currently meet active or residual criteria for any psychotic disorder, and does not meet criteria for mixed anxiety and depressive disorder symptoms.
4. **Major depressive episode superimposed on schizophrenia, schizophreniform disorder, delusional disorder, or other specified and unspecified schizophrenia spectrum and other psychotic disorder**
   - Major depressive episodes that are part of schizoaffective disorder do not meet an additional diagnosis of other specified depressive disorder.

Unspecified Depressive and Unspecified Mood Disorder

Unspecified Depressive Disorder F32.A

- This category applies to presentations in which symptoms characteristic of a depressive disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the depressive disorders diagnostic class and do not meet criteria for adjustment disorder with depressed mood or adjustment disorder with mixed anxiety and depressed mood.
- The unspecified depressive disorder category is used in situations in which the clinician chooses not to specify the reason that the criteria are not met for a specific depressive disorder, and includes presentations for which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).

Unspecified Mood Disorder F39

- This category applies to presentations in which symptoms characteristic of a mood disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not at the time of the evaluation meet the full criteria for any of the disorders in either the bipolar or the depressive disorders diagnostic classes and in which it is difficult to choose between unspecified bipolar and related disorder and unspecified depressive disorder (e.g., acute agitation).



BIPOLAR AND RELATED DISORDERS

Bipolar I, Bipolar II, and Cyclothymic Disorder

Disorder	Defining distinction
Bipolar I disorder	At least one manic episode.
Bipolar II disorder	At least one hypomanic episode and one major depressive episode, with no history of mania.
Cyclothymic disorder	Chronic fluctuations involving numerous hypomanic and depressive symptoms that do not meet full episode criteria, for the required duration.



Episode duration

- **Mania:** typically at least one week, or any duration if hospitalization is necessary.
- **Hypomania:** at least four consecutive days.

Additional Diagnostic Distinctions

Before diagnosing a mood disorder, consider:

- Severity and duration of symptoms.
- Changes from the person's usual mood and functioning.
- The presence of mania, hypomania, or mixed features.
- Psychotic symptoms, if present.
- Substance use, medications, and medical conditions.
- Cultural context and developmental stage.
- Functional impairment and safety concerns.



Clinical principle:
A diagnosis requires the complete pattern, not one isolated symptom.

Bipolar I Disorder

Diagnostic Criteria

For a diagnosis of bipolar I disorder, it is necessary to meet the following criteria for a manic episode. The manic episode may have been preceded by and may be followed by hypomanic or major depressive episodes.

Manic Episode

A. A distinct period of abnormally and persistently elevated, expansive, or irritable mood and abnormally and persistently increased activity or energy, lasting at least 1 week and present most of the day, nearly every day (or any duration if hospitalization is necessary).

B. During the period of mood disturbance and increased energy or activity, three (or more) of the following symptoms (four if the mood is only irritable) are present to a significant degree and represent a noticeable change from usual behavior:

1. Inflated self-esteem or grandiosity.
2. Decreased need for sleep (e.g., feels rested after only 3 hours of sleep).
3. More talkative than usual or pressure to keep talking.
4. Flight of ideas or subjective experience that thoughts are racing.
5. Distractibility (i.e., attention too easily drawn to unimportant or irrelevant external stimuli), as reported or observed.
6. Increase in goal-directed activity (either socially, at work or school, or sexually) or psychomotor agitation (i.e., purposeless non-goal-directed activity).
7. Excessive involvement in activities that have a high potential for painful consequences (e.g., engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments).

C. The mood disturbance is sufficiently severe to cause marked impairment in social or occupational functioning or to necessitate hospitalization to prevent harm to self or others, or there are psychotic features.

D. The episode is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication, other treatment) or another medical condition.

> **Note:** A full manic episode that emerges during antidepressant treatment (e.g., medication, electroconvulsive therapy) but persists at a fully syndromal level beyond the physiological effect of that treatment is sufficient evidence for a manic episode and, therefore, a bipolar I diagnosis.

> **Note:** Criteria A–D constitute a manic episode. At least one lifetime manic episode is required for the diagnosis of bipolar I disorder.

Hypomanic Episode

A. A distinct period of abnormally and persistently elevated, expansive, or irritable mood and abnormally and persistently increased activity or energy, lasting at least 4 consecutive days and present most of the day, nearly every day.

B. During the period of mood disturbance and increased energy and activity, three (or more) of the same symptoms (four if the mood is only irritable) have persisted, represent a noticeable change from usual behavior, and have been present to a significant degree.

C. The episode is associated with an unequivocal change in functioning that is uncharacteristic of the individual when not symptomatic.

D. The disturbance in mood and the change in functioning are observable by others.

E. The episode is not severe enough to cause marked impairment in social or occupational functioning or to necessitate hospitalization. If there are psychotic features, the episode is, by definition, manic.

F. The episode is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication, other treatment) or another medical condition.

Major Depressive Episode and Bipolar I Disorder

Major Depressive Episode

A. Five (or more) of the following symptoms have been present during the same 2-week period and represent a change from previous functioning; at least one of the symptoms is either (1) depressed mood or (2) loss of interest or pleasure.

> **Note:** Do not include symptoms that are clearly attributable to another medical condition.

1. Depressed mood most of the day, nearly every day, as indicated by either subjective report (e.g., feels sad, empty, or hopeless) or observation made by others (e.g., appears tearful). (Note: In children and adolescents, can be irritable mood.)
2. Markedly diminished interest or pleasure in all, or almost all, activities most of the day, nearly every day (as indicated by either subjective account or observation).
3. Significant weight loss when not dieting or weight gain (e.g., a change of more than 5% of body weight in a month), or decrease or increase in appetite nearly every day. (Note: In children, consider failure to make expected weight gain.)
4. Insomnia or hypersomnia nearly every day.
5. Psychomotor agitation or retardation nearly every day (observable by others, not merely subjective feelings of restlessness or being slowed down).
6. Fatigue or loss of energy nearly every day.
7. Feelings of worthlessness or excessive or inappropriate guilt (which may be delusional) nearly every day (not merely self-reproach or guilt about being sick).
8. Diminished ability to think or concentrate, or indecisiveness, nearly every day (either by subjective account or as observed by others).
9. Recurrent thoughts of death (not just fear of dying), recurrent suicidal ideation without a specific plan, or a suicide attempt or a specific plan for committing suicide.

> **Note:** Criteria A–C constitute a major depressive episode. Major depressive episodes are common in bipolar I disorder but are not required for the diagnosis of bipolar I disorder.

> **Note:** Responses to a significant loss (e.g., bereavement, financial ruin, losses from a natural disaster, a serious medical illness or disability) may include the feelings of intense sadness, rumination about the loss, insomnia, poor appetite, and weight loss noted in Criteria A, which may resemble a depressive episode. Although such symptoms may be understandable or considered appropriate to the loss, the presence of a major depressive episode in addition to the normal response to a significant loss should also be carefully considered.

Bipolar I Disorder

A. Criteria have been met for at least one manic episode (Criteria A–D under "Manic Episode").

B. At least one manic episode is not better explained by schizoaffective disorder and is not superimposed on schizophrenia, schizophreniform disorder, delusional disorder, or other specified or unspecified schizophrenia spectrum and other psychotic disorder.



Coding and Recording Procedures (Bipolar I Disorder)

The diagnostic code for bipolar I disorder is based on type of current or most recent episode and its status with respect to current severity, presence of psychotic features, and remission status. Current severity and psychotic features are only indicated if full criteria are currently met for a manic or major depressive episode. Remission specifiers are only indicated if the full criteria are not currently met for a manic, hypomanic, or major depressive episode.

Bipolar I disorder	Current or most recent episode manic	Current or most recent episode hypomanic*	Current or most recent episode depressed	Current or most recent episode unspecified**
Mild	F31.11	NA	F31.31	NA
Moderate	F31.12	NA	F31.32	NA
Severe	F31.13	NA	F31.4	NA
With psychotic features***	F31.2	NA	F31.5	NA
In partial remission	F31.73	F31.71	F31.75	NA
In full remission	F31.74	F31.72	F31.76	NA
Unspecified	F31.9	F31.9	F31.9	NA

* Severity and psychotic specifiers do not apply; code F31.0 for cases not in remission.

** Severity, psychotic, and remission specifiers do not apply. Code F31.9.

*** If psychotic features are present, code the "with psychotic features" specifier irrespective of episode severity.

Recording the Diagnosis

In recording the name of a diagnosis, terms should be listed in the following order: bipolar I disorder, type of current episode (or most recent episode if bipolar I disorder is in partial or full remission), severity/psychotic/remission specifiers, followed by as many of the following specifiers without codes as apply to the current episode (or most recent episode if bipolar I disorder is in partial or full remission).

Note: The specifiers "with rapid cycling" and "with seasonal pattern" describe the pattern of mood episodes.

Specifiers (specify if):

- **With anxious distress** (pp. 169–170)
- **With mixed features** (pp. 170–171)
- **With rapid cycling** (p. 171)
- **With melancholic features** (pp. 171–172)
- **With atypical features** (pp. 172–173)
- **With mood-congruent psychotic features** (p. 173; *applies to manic episode and/or major depressive episode*)
- **With mood-incongruent psychotic features** (p. 173; *applies to manic episode and/or major depressive episode*)
- **With catatonia** (p. 173). **Coding note:** Use additional code F06.1.
- **With peripartum onset** (pp. 173–174)
- **With seasonal pattern** (pp. 174–175)



Bipolar II Disorder (F31.81)

Diagnostic Criteria

For a **diagnosis** of bipolar II disorder, it is necessary to meet the following criteria for a **current** or past **hypomanic episode** and the following criteria for a current or past major depressive episode.

Hypomanic Episode

A. A distinct period of abnormally and persistently elevated, expansive, or irritable mood and abnormally and persistently increased activity or energy, lasting at least 4 consecutive days and present most of the day, nearly every day.

B. During the period of mood disturbance and increased energy and activity, three (or more) of the following symptoms (four if the mood is only irritable) have persisted, represent a noticeable change from usual behavior, and have been present to a significant degree:

1. Inflated self-esteem or grandiosity.
2. Decreased need for sleep (e.g., feels rested after only 3 hours of sleep).
3. More talkative than usual or pressure to keep talking.
4. Flight of ideas or subjective experience that thoughts are racing.
5. Distractibility (i.e., attention too easily drawn to unimportant or irrelevant external stimuli), as reported or observed.
6. Increase in goal-directed activity (either socially, at work or school, or sexually) or psychomotor agitation.
7. Excessive involvement in activities that have a high potential for painful consequences (e.g., engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments).

Hypomanic Episode (cont'd)

C. The episode is associated with an unequivocal change in functioning that is uncharacteristic of the individual when not symptomatic.

D. The disturbance in mood and the change in functioning are observable by others.

E. The episode is not severe enough to cause marked impairment in social or occupational functioning or to necessitate hospitalization. If there are psychotic features, the episode is, by definition, manic.

F. The episode is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication, other treatment) or another medical condition.

Note: A full hypomanic episode that emerges during antidepressant treatment (e.g., medication, electroconvulsive therapy) but persists at a fully syndromal level beyond the physiological effect of that treatment is sufficient evidence for a hypomanic episode diagnosis. However, caution is indicated so that one or two symptoms (particularly increased irritability, edginess, or agitation following antidepressant use) are not taken as sufficient for diagnosis of a hypomanic episode, nor necessarily indicative of a bipolar diathesis.

Major Depressive Episode

A. Five (or more) of the following symptoms have been present during the same 2-week period and represent a change from previous functioning; at least one of the symptoms is either (1) depressed mood or (2) loss of interest or pleasure.

Note: Do not include symptoms that are clearly attributable to a medical condition.

1. Depressed mood most of the day, nearly every day, as indicated by either subjective report (e.g., feels sad, empty, or hopeless) or observation made by others (e.g., appears tearful). (Note: In children and adolescents, can be irritable mood.)
2. Markedly diminished interest or pleasure in all, or almost all, activities most of the day, nearly every day (as indicated by either subjective account or observation).



Bipolar II Disorder (F31.81)

Diagnostic Criteria

A. Criteria have been met for at least one hypomanic episode (Criteria A–F under “Hypomanic Episode”) and at least one major depressive episode (Criteria A–C under “Major Depressive Episode”).

B. There has never been a manic episode.

C. At least one hypomanic episode and at least one major depressive episode are not better explained by schizoaffective disorder and are not superimposed on schizophrenia, schizophreniform disorder, delusional disorder, or other specified or unspecified schizophrenia spectrum and other psychotic disorder.

D. The symptoms of depression or the unpredictability caused by frequent alternation between periods of depression and hypomania cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Coding and Recording Procedures

Bipolar II disorder has one diagnostic code: **F31.81.** Its status with respect to current severity, presence of psychotic features, course, and other specifiers cannot be coded but should be indicated in writing (e.g., F31.81 bipolar II disorder, current episode depressed, moderate severity, with mixed features; F31.81 bipolar II disorder, most recent episode depressed, in partial remission).

Specify current or most recent episode:

- **Hypomanic**
- **Depressed**

If current episode is hypomanic (or most recent episode if in partial or full remission):

Specify if:

- With anxious distress (pp. 169–170)
- With mixed features (pp. 170–171)
- With rapid cycling (p. 171)
- With peripartum onset (pp. 173–174)
- With seasonal pattern (pp. 174–175)



Cyclothymic Disorder (F34.0)

Diagnostic Criteria

A. For at least 2 years (at least 1 year in children and adolescents) there have been numerous periods with hypomanic symptoms that do not meet criteria for a hypomanic episode and numerous periods with depressive symptoms that do not meet criteria for a major depressive episode.

B. During the above 2-year period (1 year in children and adolescents), Criterion A symptoms have been present for at least half the time and the individual has not been without the symptoms for more than 2 months at a time.

C. Criteria for a major depressive, manic, or hypomanic episode have never been met.

D. The symptoms in Criterion A are not better explained by schizoaffective disorder, schizophrenia, schizophreniform disorder, delusional disorder, or other specified or unspecified schizophrenia spectrum and other psychotic disorder.

E. The symptoms are not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Specify if:

With anxious distress (see pp. 169–170)



Substance/Medication-Induced Bipolar and Related Disorder

Diagnostic Criteria

A. A prominent and persistent disturbance in mood that predominates in the clinical picture and is characterized by abnormally elevated, expansive, or irritable mood and abnormally increased activity or energy.

B. There is evidence from the history, physical examination, or laboratory findings of both:

1. The symptoms in Criterion A developed during or soon after substance intoxication or withdrawal or after exposure to or withdrawal from a medication.
2. The involved substance/medication is capable of producing the symptoms in Criterion A.

C. The disturbance is not better explained by a bipolar or related disorder that is not substance/medication-induced. Such evidence could include the symptoms preceding the onset of substance/medication use, persistence of symptoms for a substantial period (e.g., about 1 month) after cessation of withdrawal or intoxication, or a history of recurrent non-substance/medication-related episodes.

D. The disturbance does not occur exclusively during the course of a delirium.

E. The symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

Specify (see Table 1):

- With onset during intoxication
- With onset during withdrawal
- With onset after medication use

Coding and Recording Procedures

The ICD-10-CM codes depend on whether a comorbid substance use disorder is present for the same class of substance. If a mild substance use disorder is comorbid, use the 4th position character "1"; if moderate or severe, use "2." If there is no comorbid substance use disorder, use "9." A separate diagnosis of substance use disorder is not given.

ICD-10-CM Codes

Substance/Medication	With mild use disorder	With moderate or severe use disorder	Without use disorder
Alcohol	F10.14	F10.24	F10.94
Phencyclidine	F16.14	F16.24	F16.94
Other hallucinogen	F16.14	F16.24	F16.94
Sedative, hypnotic, or anxiolytic	F13.14	F13.24	F13.94
Amphetmaine-type substance (or other stimulant)	F15.14	F15.24	F15.94
Cocaine	F14.14	F14.24	F14.94
Other (or unknown) substance	F19.14	F19.24	F19.94

Note: This diagnosis should be made instead of a diagnosis of substance intoxication or withdrawal only when the symptoms in Criterion A predominate in the clinical picture and are sufficiently severe to warrant clinical attention.

Bipolar and Related Disorder Due to Another Medical Condition

Diagnostic Criteria

A. A prominent and persistent disturbance in mood that predominates in the clinical picture and is characterized by abnormally elevated, expansive, or irritable mood and abnormally increased activity or energy.

B. There is evidence from the history, physical examination, or laboratory findings that the disturbance is the direct pathophysiological consequence of another medical condition.

C. The disturbance is not better explained by another mental disorder.

D. The disturbance does not occur exclusively during the course of a delirium.

E. The disturbance causes clinically significant distress or impairment in social, occupational, or other important areas of functioning, or necessitates hospitalization to prevent harm to self or others, or there are psychotic features.

Coding note: The ICD-10-CM code depends on the specifier (see below).

Specify if:

- **F06.33 With manic features:** Full criteria are not met for a manic or hypomanic episode.
- **F06.33 With manic- or hypomanic-like episode:** Full criteria are met except Criterion D for a manic episode or except Criterion F for a hypomanic episode.
- **F06.34 With mixed features:** Symptoms of depression are also present but do not predominate in the clinical picture.

Coding note: Include the name of the other medical condition in the name of the bipolar and related disorder (e.g., F06.33 bipolar disorder due to hyperthyroidism, with manic features). The other medical condition should also be coded and listed separately immediately before the bipolar and related disorder due to the medical condition (e.g., E05.90 hyperthyroidism; F06.33 bipolar disorder due to hyperthyroidism, with manic features).



Other Specified Bipolar and Related Disorder

F31.89

This category applies to presentations in which symptoms characteristic of a bipolar and related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the bipolar and related disorders diagnostic class. The other specified bipolar and related disorder category is used in situations in which the clinician chooses to communicate the specific reason that the presentation does not meet the criteria for any specific bipolar and related disorder. This is done by recording "other specified bipolar and related disorder" followed by the specific reason (e.g., "short-duration cyclothymia").

Examples of presentations that can be specified using the "other specified" designation include the following:

1. **Short-duration hypomanic episodes (2–3 days) and major depressive episodes:** A lifetime history of one or more major depressive episodes in individuals whose presentation has never met full criteria for a manic or hypomanic episode but who have experienced two or more episodes of short-duration hypomania that meet the full symptomatic criteria for a hypomanic episode but that only last for 2–3 days. The episodes of hypomanic symptoms do not overlap in time with the major depressive episodes, so the disturbance does not meet criteria for major depressive episode, with mixed features.

2. **Hypomanic episodes with insufficient symptoms and major depressive episodes:** A lifetime history of one or more major depressive episodes in individuals whose presentation has never met full criteria for a manic or hypomanic episode but who have experienced one or more episodes of hypomania that do not meet full symptomatic criteria (i.e., at least 4 consecutive days of elevated mood and one or two of the other symptoms of a hypomanic episode, or irritable mood and two or three of the other symptoms of a hypomanic episode). The episodes do not overlap in time with the major depressive episodes, so the disturbance does not meet criteria for major depressive episode, with mixed features.

3. **Hypomanic episode without prior major depressive episode:** One or more hypomanic episodes in an individual whose presentation has never met full criteria for a major depressive episode or a manic episode.

4. **Short-duration cyclothymia (less than 24 months):** Multiple episodes of hypomanic symptoms that do not meet criteria for a hypomanic episode and multiple episodes of depressive symptoms that do not meet criteria for a major depressive episode that persist over a period of less than 24 months (less than 12 months for children or adolescents) in an individual whose presentation has never met full criteria for a major depressive, manic, or hypomanic episode and does not meet criteria for any psychotic disorder. During the course of the disorder, the hypomanic or depressive symptoms are present for more days than not, the individual has not been without symptoms for more than 2 months at a time, and the symptoms cause clinically significant distress or impairment.

5. **Manic episode superimposed on schizophrenia, schizophreniform disorder, delusional disorder, or other specified and unspecified schizophrenia spectrum and other psychotic disorder.**

   Note: Manic episodes that are part of schizoaffective disorder do not merit an additional diagnosis of other specified bipolar and related disorder.



Unspecified Bipolar and Related Disorder F31.9

This category applies to presentations in which symptoms characteristic of a bipolar and related disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not meet the full criteria for any of the disorders in the bipolar and related disorders diagnostic class. The unspecified bipolar and related disorder category is used in situations in which the clinician chooses *not* to specify the reason that the criteria are not met for a specific bipolar and related disorder, and includes presentations in which there is insufficient information to make a more specific diagnosis (e.g., in emergency room settings).

Unspecified Mood Disorder F39

This category applies to presentations in which symptoms characteristic of a mood disorder that cause clinically significant distress or impairment in social, occupational, or other important areas of functioning predominate but do not at the time of the evaluation meet the full criteria for any of the disorders in either the bipolar or the depressive disorders diagnostic classes and in which it is difficult to choose between unspecified bipolar and related disorder and unspecified depressive disorder (e.g., acute agitation).



Prevalence Across the Life Span

Mood disorders can occur at different stages of life.

Children and adolescents	• Symptoms may involve irritability, withdrawal, academic difficulties, or changes in behavior.
Adults	• Episodes may affect work, relationships, responsibilities, and physical health.
Older adults	• Mood symptoms may coexist with medical conditions, bereavement, medication effects, or cognitive concerns.

Assessment should be developmentally appropriate.

Mood Disorders Across Cultures and Among Creative Individuals

Culture and mood disorders

- Cultural norms influence how people describe emotional distress.
- Some individuals emphasize physical symptoms rather than sadness.
- Stigma can affect disclosure and help-seeking.
- Cultural context should inform assessment without replacing diagnostic evidence.

Mood disorders and creativity

- Historical accounts describe mood symptoms among some creative individuals.
- Associations do not establish that mood disorders cause creativity.
- Most people with mood disorders should not be stereotyped as unusually creative or dangerous.

Biological Dimensions

Biological contributions may include:

- Genetic vulnerability and family history.
- Differences in neural circuits involved in emotion and reward.
- Neurotransmitter systems involved in mood regulation.
- Circadian rhythms and sleep-wake regulation.
- Stress-related physiological processes.



Important:
No single gene, neurotransmitter, or brain region explains all mood disorders.



Brain Structure, Function, and Psychological Dimensions

Brain and behavioral processes



- Emotion regulation and cognitive control.
- Reward processing and motivation.
- Stress reactivity and sleep regulation.

Psychological contributions

- Negative interpretations of events.
- Rumination and repetitive negative thinking.
- Learned patterns of avoidance or withdrawal.
- Reduced engagement in rewarding activities.
- Difficulties coping with stress.

Vulnerability develops through interacting processes.

Social and Cultural Dimensions: An Integrative Theory

Social and environmental influences

- Relationship conflict and loss.
- Chronic stress and financial difficulties.
- Social isolation and discrimination.
- Family and community support.
- Cultural expectations and barriers to care.

Biopsychosocial integration

Biological vulnerability + psychological processes + social context can interact over time.

Mood disorders are multifactorial, not signs of personal weakness.

Medications and Brain Stimulation

Medication approaches

- Antidepressants may be used for depressive disorders.
- Mood stabilizers are important in bipolar disorder.
- Some antipsychotic medications are used in selected depressive or bipolar presentations.

Brain stimulation approaches

- Electroconvulsive therapy (ECT): an established treatment for selected severe or treatment-resistant presentations.
- Repetitive transcranial magnetic stimulation (rTMS): a noninvasive treatment used for selected depressive presentations.

Treatment choice depends on diagnosis, severity, history, risks, and patient preference.

Psychological and Combined Treatments for Depression

Cognitive Behavioral Therapy (CBT)

- Identifies and evaluates unhelpful thinking patterns.
- Develops more adaptive coping and problem-solving skills.

Behavioral Activation

- Gradually increases meaningful and rewarding activities.
- Targets withdrawal and reduced reinforcement.

Interpersonal Psychotherapy (IPT)

- Addresses relationship difficulties, role transitions, grief, and interpersonal stress.

Combined treatment

- Psychotherapy and medication may be combined when clinically appropriate.

Treatment for Bipolar Disorder and Relapse Prevention

Bipolar disorder management

- Appropriate medication management.
- Psychoeducation about mood episodes.
- Interpersonal and Social Rhythm Therapy (IPSRT).
- Family-focused interventions.
- Monitoring sleep, energy, and mood changes.

Relapse prevention

- Recognize early warning signs.
- Maintain a consistent sleep-wake routine.
- Follow the agreed treatment plan.
- Attend follow-up appointments.
- Seek help promptly when symptoms change.

Never discontinue prescribed medication without consulting the treating clinician.

Suicide: Statistics and Public Health Significance

Suicide is a preventable public health concern.

- More than 720,000 people die by suicide worldwide each year, according to WHO estimates.
- In 2021, suicide was the third leading cause of death among people aged 15–29 globally.
- Suicide affects individuals, families, schools, workplaces, and communities.
- Suicide risk is influenced by multiple interacting factors, not one single cause.



Prevention requires early identification, access to care, and coordinated community support.

Source: World Health Organization (2025).

Causes, Risk Factors, and Protective Factors

Potential risk factors

- Previous suicide attempts or suicidal behavior.
- Depression, bipolar disorder, substance-related problems, or other mental health difficulties.
- Severe emotional distress, hopelessness, or social isolation.
- Relationship conflict, bereavement, violence, or abuse.
- Financial difficulties, chronic pain, or major life stressors.
- Access to highly lethal means during a crisis.

Protective factors

- Supportive relationships and a sense of belonging.
- Access to mental health and medical care.
- Effective coping and problem-solving skills.
- Reasons for living and meaningful social roles.
- Reduced access to lethal means during periods of risk.
- A collaborative safety and follow-up plan.

Is Suicide Contagious?



Suicide contagion

Exposure to suicide-related behavior or reporting may contribute to increased risk among some vulnerable individuals.

Responsible communication

- Avoid sensationalized or romanticized portrayals.
- Do not describe suicide methods or provide unnecessary details.
- Avoid presenting suicide as an inevitable response to a problem.
- Emphasize help-seeking, recovery, and available support.
- Respond sensitively to people affected by suicide loss.

Key message: Responsible communication is part of suicide prevention.

Suicide Assessment, Intervention, and Treatment

When someone may be at risk

1. Ask directly and compassionately about suicidal thoughts.
2. Assess immediate safety, including current thoughts, intent, plans, access to means, and relevant history.
3. Listen without judgment and acknowledge the person's distress.
4. Develop a collaborative safety plan with appropriate professional support.
5. Arrange referral and follow-up with qualified mental health or medical professionals.
6. Escalate to emergency support when there is immediate danger.



Never promise secrecy when someone's safety is at serious risk.

PART VII — APPLICATION AND ASSESSMENT

Virtual Case Activity: Formulate, Don't Assume

CASE A — Persistent Low Mood

A college student reports low mood, loss of interest, fatigue, difficulty concentrating, and feelings of worthlessness over several weeks. Academic and social functioning have declined.

CASE B — Unusually Elevated Mood

A young professional has experienced a distinct period of unusually elevated and irritable mood, very little need for sleep, rapid speech, inflated confidence, and increasingly risky decisions. The change has caused serious disruption.

CASE C — Warning Signs

A student who has recently experienced a major loss begins withdrawing from friends, expressing hopelessness, and saying that others would be better off without them.

Group task

1. Identify the most relevant clinical concern.
2. State what additional information is needed.
3. Recommend an appropriate next step.

Homework: Biopsychosocial Case Reflection

Individual assignment:
Understanding Mood Disorders and Suicide Prevention

Write a 400–500-word case reflection using a fictional individual with a possible mood disorder.

Include:

1. The observed symptoms and possible diagnostic considerations.
2. Biological, psychological, and social factors that may contribute to the presentation.
3. One or more appropriate evidence-informed treatment approaches.
4. Relevant risk and protective factors, if applicable.
5. An ethical recommendation for assessment, referral, or support.

Submission:
Next class meeting

Format:
PDF or Word document

References:
At least two credible academic or professional sources in APA 7th edition format.





References

APA 7th Edition

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). https://doi.org/10.1176/appi.books.9780890425787

Barlow, D. H., & Durand, V. M. (2015). *Abnormal psychology: An integrative approach* (7th ed.). Cengage Learning.

Cuijpers, P., Quero, S., Noma, H., Ciharova, M., Miguel, C., Karyotaki, E., Cipriani, A., Cristea, I. A., & Furukawa, T. A. (2021). Psychotherapies for depression: A network meta-analysis covering efficacy, acceptability and long-term outcomes of all main treatment types. *World Psychiatry, 20*(2), 283–293. https://doi.org/10.1002/wps.20860

World Health Organization. (2025). *Suicide: Facts and figures globally*. https://www.who.int/publications/i/item/9789240110069